BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm that either has happened.

Check	Before starting	First treatment block	During treatment	Change / completion	Follow-up
Blood / kidney / liver	FBC, renal and liver profile	Day 1: FBC, renal and liver tests	Day 8: FBC; review both doses	Review unresolved abnormalities	Individual clinical plan
Heart / breathing	Review symptoms and prior treatment	Investigate clinical concerns	No routine HER2-style echo schedule	Review new chest/breathing symptoms	Clinical indication
Cancer review	Confirm stage and setting	Agree review checkpoints	Review symptoms and response	Discuss continuation / next treatment	Setting-specific review
Other checks	Review previous treatment	Review support medicines	Review toxicity before each dose	Endocrine / bone plan where relevant	Follow unresolved symptoms

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00430 v6 · official regimen · NICE NG101 · bone health and follow-up · ASCO · biomarkers in metastatic breast cancer · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

Gemcitabine + carboplatin AUC2: treatment and checks

Locally recurrent or metastatic triple-negative breast cancer

Carboplatin AUC2 is given first, followed by gemcitabine 1,000 mg/m², on BOTH Days 1 and 8 of each 21-day cycle. Treatment continues until progression or unacceptable toxicity.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment block	During treatment	Change / completion	Follow-up
Cancer medicines NCCP 00430	Confirm exact regimen	Gemcitabine + carboplatin AUC2	Carboplatin AUC2 is given first, followed by gemcitabine 1,000 mg/m², on BOTH Days 1 and 8 of each 21-day cycle. Treatment continues until progression or unacceptable toxicity.	Review active medicines and purpose	Next treatment / follow-up plan
Surgery / radiotherapy NCCP 00430	Locally recurrent or metastatic triple-negative breast cancer	Follow the MDT sequence	Radiotherapy timing needs agreement	Selected local treatment as indicated	Review recovery and ongoing effects
Blood / kidney / liver NCCP 00430	FBC, renal and liver profile	Day 1: FBC, renal and liver tests	Day 8: FBC; review both doses	Review unresolved abnormalities	Individual clinical plan
Heart / breathing NCCP 00430	Review symptoms and prior treatment	Investigate clinical concerns	No routine HER2-style echo schedule	Review new chest/breathing symptoms	Clinical indication
Cancer review NCCP 00430 · METASTATIC · MARKERS	Confirm stage and setting	Agree review checkpoints	Review symptoms and response	Discuss continuation / next treatment	Setting-specific review
Other checks NCCP 00430 · BONE	Review previous treatment	Review support medicines	Review toxicity before each dose	Endocrine / bone plan where relevant	Follow unresolved symptoms

What each check is for

1 Baseline review

Before treatment

Locally recurrent or metastatic triple-negative breast cancer. There is no routine three-month echo schedule for CMF or gemcitabine alone. Review prior treatment and cardiac risk; new chest pain or breathing symptoms need urgent assessment.

Ask / act: Confirm source eligibility and results.

Sources: NCCP 00430

2 Treatment-day check

Before each planned dose

Baseline and Day 1: full blood count, kidney and liver tests. Day 8: full blood count. Review kidney-function dosing and whether either medicine is held before each administration.

Ask / act: Confirm active medicines and whether a dose is held.

Sources: NCCP 00430

3 Support plan

At every phase or day change

Carboplatin AUC2 is given first, followed by gemcitabine 1,000 mg/m², on BOTH Days 1 and 8 of each 21-day cycle. Treatment continues until progression or unacceptable toxicity.

Ask / act: Choose the matching support script; do not merge phases.

Sources: NCCP 00430

4 Response and follow-up

Agreed review points

For metastatic disease, record the disease sites and agree the scan type and review interval before treatment. Review response, symptoms and toxicity together; worsening symptoms may need earlier assessment.

Ask / act: Review symptoms, results and patient priorities together.

Sources: NCCP 00430 · METASTATIC · MARKERS

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-25 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.18